

Derbyshire Shared Care Pathology Guidelines

Thrombocytosis in adults

Introduction

Thrombocytosis is present in 1.5 - 2.2% of the primary care consulting population ≥40 years

Important

Very high platelet counts ($>1000 \times 10^9/L$) in the setting of myeloproliferative disorders carry risk of both thrombosis and abnormal bleeding (due to platelet dysfunction)



The laboratory usually telephones platelet results $>1000 \times 10^9/L$ to GP practices when open or to the GP out of hours services when the GP practice is closed.

Causes of Thrombocytosis (*main causes in italics*)

Primary	Secondary
• Essential thrombocythaemia • Polycythaemia vera • Primary myelofibrosis • Myelodysplasia with del (5q) • Refractory anaemia with ring sideroblasts associated with marked thrombocytosis • Chronic myeloid leukaemia • Chronic myelomonocytic leukaemia • Atypical chronic myeloid leukaemia • MDS/MPN-U (Myelodysplastic syndrome / Myeloproliferative neoplasm - Unclassifiable)	• <i>Infection</i> • <i>Inflammation</i> • <i>Post-operative</i> • <i>Iron deficiency</i> • <i>Malignancy</i> • High BMI/metabolic syndrome • Haemorrhage • Haemolysis • Hyposplenism • Drug therapy (corticosteroids, adrenaline) • Rebound following chemotherapy

Symptoms

- Patients are usually asymptomatic
- Symptoms may include headache, dizziness, light headedness, vision changes such as blurred vision, easy bruising or bleeding, tingling in the hands and feet
- In secondary thrombocytosis, signs and symptoms are related to the underlying condition

What to do next

- A detailed history and examination is required to distinguish potential primary and secondary causes see NICE CKS [Assessment of thrombocytosis | Diagnosis | Platelets - abnormal counts and cancer | CKS | NICE](#)
- Look for and treat secondary causes (see table on page 1)
 - FBC, CRP, Ferritin, TSAT
 - Patients that are iron deficient should have iron replacement unless they have polycythaemic indices.
 - Re check platelet count when iron replete and action if remains raised.
 - Consider NICE (NG12) investigations as appropriate (chest X-ray if >40yr, pelvic USS in women >55yr, FIT test in iron deficient anaemia, upper GI endoscopy if >55yr with vomiting / nausea)

Referral Criteria

Urgent Haematology referral (not 2WW)

- Unexplained Platelet count $>1000 \times 10^9/L$
- Platelet Count $450-1000 \times 10^9/L$ associated with:
 - Recent (within 3 months) stroke, DVT or PE, MI / NSTEMI or other acute arterial thrombotic event with persistent thrombocytosis
 - Splenomegaly (palpable spleen) – 2WW is appropriate in this case

Routine Haematology referral

- Platelet count $> 450 \times 10^9/L$, persistent and unexplained for 6-8 weeks, with no secondary cause and normal iron stores.

Contacts

Urgent queries:

UHDB Haematology doctor on call
CRH Consultant Haematologist

Via RDH/QHB switchboard
Via Consultant Connect or on-call Consultant
through CRH switchboard

Non urgent queries:

UHDB Consultant Haematologist
CRH Consultant Haematologist

01332 787973 (9am – 5pm)
crhft.cabadviceandguidance@nhs.net

References

1. Suspected cancer: recognition and referral (NICE, NG12)
<https://www.nice.org.uk/guidance/ng12>
2. Sarah ER Bailey et al. Clinical relevance of thrombocytosis in primary care: a prospective cohort study of cancer incidence using English electronic medical records and cancer registry data. Br J Gen Pract 2017; 67 (659): e405-e413
3. Claire N. Harrison et al. Guideline for investigation and management of adults and children presenting with a thrombocytosis.
<https://onlinelibrary.wiley.com/doi/10.1111/j.1365-2141.2010.08122.x>

4. Essential Thrombocytosis (Canadian Cancer Society)
<http://www.cancer.ca/en/cancer-information/cancer-type/leukemia/leukemia/essential-thrombocytosis/?region=on>
5. NICE CKS How should I assess a person with thrombocytosis? Updated Dec 2025

Document controls

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Approval	Date	Name(s)	Designation
Secondary Care UHDB	06.03.26	Dr F Al Kaisi, Dr P Dyer	Consultant Haematologists
Secondary Care CRH	06.03.26	Dr J Tam, Dr K Lam	Consultant Haematologists
Primary Care	06.03.26	Dr P Blackwell	GP
Laboratory (Derbyshire Pathology)	06.03.26	Ms S Knowles	Consultant Clinical Scientist